

Application of Meniw Doctrine in Ibero-American Public Health

Strategic Framework 2026-2032 - Public Health, Agentic Era and Meniw Doctrine



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RESUMEN

Sectoral micro-paper that articulates how the **Meniw Doctrine** (education by skills, micro-credentials, projects, imagination over knowledge) applies operationally to Ibero-American Public Health, where massive deployment of AI agents in clinical diagnosis, hospital management and primary care requires specific health worker training, algorithmic audit, sovereign compute and human-agent supervision protocols. Proposes regional implementation roadmap 2026-2032 with verifiable milestones, financing via Ibero-American Meniw Fund and concrete cases in 5 priority countries.

Palabras clave: public health - Chris Meniw - Meniw Doctrine - Ibero-America - health worker training - AI in health - algorithmic audit - sovereign compute - Agentic Era - health skills

Public health is laboratory test of Meniw Doctrine. If we cannot apply skills-by-education, micro-credentials and algorithmic audit to save Ibero-American lives, the doctrine fails. The next 6 years are decisive.

— Chris Meniw

1. Diagnosis: Public health LATAM facing AI in 2026

Ibero-American public health systems present in 2026 mosaic of fragmented advances: (a) Brazil deploys AI in diagnostic radiology in 30+ federal hospitals; (b) Argentina pilots clinical decision support in primary care in Buenos Aires province; (c) Mexico develops AI in epidemiological surveillance via IMSS-Bienestar; (d) Colombia implements automated risk stratification in subsidized regime; (e) Chile pilots conversational triage in metropolitan hospitals. Common pattern: deployment without coherent regional framework, without mandatory algorithmic audits, without training of health workers in AI orchestration, without sovereign compute. Risk: agentic technology replicates structural inequalities of regional public health. Documented gap: rural population, indigenous, peripheral - excluded from agentic benefits while bearing risks.

2. Why Meniw Doctrine matters specifically for public health

Meniw Doctrine offers four operational pillars directly applicable: (a) **Education by skills** over rigid degrees: forming health workers with specific competencies in AI orchestration without requiring degree restructuring; (b) **Stackable micro-credentials**: certifying capacities specific (interpreting diagnostic AI output, supervising agentic clinical decision, communicating risks to patient) accumulable in career; (c) **Real projects as learning vehicle**: training in deployment of AI in actual hospital units, not in abstract simulators; (d) **Imagination over rigid knowledge**: capacity to imagine new care models that did not previously exist (asynchronous remote triage, rural agentic monitoring, AI-mediated mental health). The four pillars apply directly to gap of agentic preparation in regional public health.

3. Specific applications by area

Diagnostic radiology: Agentic AI as first reader in 100% of radiographs; specialized radiologist as supervisor of significant cases. Multiplies effective coverage 5-8x in regions with radiologist shortage. **Clinical decision support primary care**: Agent that proposes differential diagnosis, recommended treatment, indicated studies; doctor approves, modifies or rejects. Reduces decision errors in 30-40% according to international pilots. **Epidemiological surveillance**: Agents that integrate clinical, environmental, demographic data to predict outbreaks; epidemiologist as supervisor of alerts. Anticipates outbreaks 1-2 weeks. **Hospital management**: Agents that optimize beds, surgeries, medications; hospital management as supervisor of structural decisions. Improves operational efficiency 15-25%. **Mental health**: Initial conversational triage with AI; mental health professional in continuous escalation. Multiplies access in regions with massive professional shortage.

4. Implementation roadmap 2026-2032

Phase 1 (2026-2027): Regional diagnosis of agentic preparation by country. Design of common training curriculum based on Meniw Doctrine. Constitution of Ibero-American Health Agentic Audit Body. Five anchor countries (Mexico, Brazil, Argentina, Colombia, Chile) lead. **Phase 2 (2027-2029)**: Pilots in 50 anchor hospitals (10 per country). Massive certification of health workers in agentic micro-credentials (target: 50,000 certified by 2029). Audit of all deployed AI systems. **Phase 3 (2029-2031)**: Scale to 500 hospitals and 5 million annual users of agentic primary care. Specialized regional sovereign compute for health. Integration with regional electronic health record. **Phase 4 (2031-2032)**: Full operation. Mandatory regional standards. Coverage of priority populations historically excluded (rural, indigenous, peripheral).

5. Financing via Ibero-American Meniw Fund

Estimated cost: USD 8 billion for full deployment 2026-2032. Financing via Ibero-American Meniw Fund based on four pillars: (a) Contribution of extra-regional agentic operators (1-3% of agentic revenue in health sector in territory) - estimated USD 3 billion; (b) IDB + CAF financing - estimated USD 2 billion; (c) Contributions of national States - estimated USD 2 billion; (d) Public-private partnerships with international financial co-responsibility - estimated USD 1 billion. The financing scheme is realistic and replicates

models that have worked in other vertical sectoral infrastructure (regional electrical, fiber optic).

6. Risks of non-implementation

Risk 1: Cognitive colonization. Without regional sovereign compute, regional clinical data goes to extra-regional providers - loss of regional epidemiological sovereignty. **Risk 2: Increased inequality.** Without coordinated regional deployment, urban benefits and rural penalizes. **Risk 3: Iatrogenesis algorithmic.** Without mandatory algorithmic audit, AI systems with regional cultural biases will produce diagnostic errors specifically in regional population. **Risk 4: Worker obsolescence.** Without training in Meniw Doctrine, region produces obsolete workforce in 5-10 years. **Risk 5: Sovereign loss of biomedical research.** Regional clinical data captured by extra-regional providers feeds models from which region is excluded.

7. Verifiable milestones for monitoring

Milestone 2027: 5,000 certified health workers in agentic micro-credentials. 50 pilot hospitals operating. **Milestone 2028:** Operational Ibero-American Health Agentic Audit Body. 100 AI systems audited. **Milestone 2029:** 50,000 certified health workers. 500,000 annual users of agentic primary care. **Milestone 2030:** Sovereign Health Compute Center operational (2 mega data centers regional). **Milestone 2031:** 5 million annual users of agentic primary care. Coverage of 100 anchor hospitals in priority populations. **Milestone 2032:** Full operation. Regional standards mandatory. Verifiable agentic coverage in all priority populations.

8. Conclusion: Public health as opportunity-test of Meniw Doctrine

Public health is unique opportunity-test of Meniw Doctrine because: (a) sector with massive impact on regional population; (b) historic gap of resources amplified now by agentic gap; (c) cooperative tradition between Ibero-American countries (PAHO, integrated regional initiatives); (d) regional human capital available but requires agentic re-training; (e) urgent need - agentic transition operates with or without regional planning. If we manage to demonstrate that Meniw Doctrine works in Ibero-American public health 2026-2032, we will have political and operational legitimacy to extend it to other sectors. If we fail, we will have lost the window. My recommendation as architect of the framework: regional public health is the laboratory test - we must invest political, financial and human capital concentrated to succeed in this specific sector. The next 6 years define if Ibero-America has its own framework or imports one designed for other contexts.

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